A SHOT IN THE ARM FOR PATIENTS WITH ACID PEPTIC DISORDERS (GERD, PEPTIC ULCER)



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Gastroenterologist and Hepatologist Dr. (Prof) Anil Arora, a celebrated doctor known for Excellence in Hepatology for his immense innovative contributions in academics and research talks in this exclusive interview about the advancement in treatment for acid peptic diseases. He delves into the details of Vonoprazan as a new kid on the block with immense promise for the management of acid peptic disorders. Dr. (Prof) Anil Arora's patient-centric approach through interventional and therapeutic Gastroenterology care and services has brought succour to thousands of lives. As he continues to follow his passion of enhancing the field of Liver Gastroenterology & Pancreatico Biliary Sciences, he talks here about stomach ulcers and its future course of treatment.

For the benefit of the readers, we wish you to begin by telling us what stomach ulcers are and how are they caused?

Peptic ulcers are a group of disorders in the stomach that are caused by excessive acid production. The incidence and prevalence of this disease is rapidly increasing as one third of Indian population suffers from it This is triggered by stress, ingestion of drugs such as non-steroidal anti-inflammatory drugs, comorbidities such as coronary artery disease, chronic obstructive disease or chronic kidney disease. Excessive acid degrades the protective internal lining of the stomach called the mucous layer leading to gastro-duodenal ulcers. This can lead to either obstruction to the outflow tract of the stomatch called gastric outlet obstruction or else it can precipitate bleeding from the stomatch or duodenum, resulting in vomiting of the blood.

What are the common symptoms of ulcers that should be a red flag for anyone?

Acidity usually presents in the form of pain and discomfort in the upper part of the abdomen, below the rib cage or burning sensation that gets aggravated by spicy meals and smoking and if the stomach tends to have a lot of acid it moves up into the food pipe. Normally there is a tight competent valve between the oesophagus and stomach ,if it becomes loose or incompetent then the food rather than travelling down into the intestine for digestion ,tends to move up into the food pipe and result in troublesome heartburn. The large amount of acid acid , can travel up in the throat, and may result in hoarness of the voice, chocking sensation or lead to chronic cough especially at night. When the acid climbs up to the oropharynx it may lead to dental erosions or even dental caries.

Can you tell us something about the anti-ulcer medications?

Large number of drugs are available for treatment of peptic ulcer disease. Anti-ulcer medications are of two categories: First group of the drugs are mucoprotective agents. These drugs like Sucralfate, Alginate tend to coat the disrupted epithelium of the stomach and the oesophagus so that the acid's damaging effect is counterbalanced. Also, second group of drugs that tend to specifically decrease the acid production in the stomach, are known as either proton pump inhibitors (PPIs) or H2 Receptor Antagonists (H2RA).

Readers would like to know the advancements in drugs for ulcers?

First group of drugs for suppression of acid was launched in the late 1970s called H2RA (e.g., Cimetidine). Subsequently, Famotidine, Ranitidine,Nizetidine, came into the market unfortunately these were all were weak acid suppressants. Final frontiers for neutralising the acid in the stomach came with the discovery of new class of drugs called PPIs. PPIs such as omeprazole, lansoprazole, esomeprazole, pantoprazole ,Rabeprazole etc. go into the gastric lumen, get attached to the with the proton pumps of the parietal cells of the stomach(Acid producing apparatus) and inactive it. For almost 40 years after the discovery of the first generation PPIs that is omeprazole ,PPIs have ruled the roost in terms of usage and safety. Recently, a lot of debate regarding the side effects of long-term use of PPIs ,like bone and renal problems ,is taking place in medical community, especially concerning the first generation drugs mentioned above

There is a new hope with the Vonoprazan drug? Can you explain?

The new drug called vonoprazan, which was first discovered by the Japanese scientists, has four advantages over conventionally available PPIs. Unlike the first generation of PPIs, vonoprazan can be given before or after food and Vonoprazan acts more rapidly than the conventionally available PPI.

The biggest advantage of vonoprazan over other PPIs is that it will try to suppress both active as well as inactive proton pumps. So, the silent and the dormant proton pumps, which were typically not affected by the PPIs will be blocked by vonoprazan, giving 10% more advantage in acid suppression. Most importantly, Vonoprazan is not metabolised through CYP2C19, which means it has minimal interaction with other drugs. This may be distinct advantage especially because a large number of our patients are typically taking multiple drugs for treatment of diabetes, heart illness, dyslipidemia leading to a number of drug - drug interactions. This new drug, vonoprazan has shown that even in patients with severe reflux esophagitis or for eradication H.pylori, the bacteria touted to be responsible for more than 90% of the ulcers it has the advantages over and above the conventional PPIs.

What is it that is positive for patients with Vonoprazan treatment?

Vonoprazan is a good drug with a bright future. Its long-term use will further confirm its safety and efficacy as it has already been shown in Japan, where the drug is in use for almost 10 years. Vonoprazan is safer, is a more potent acid suppressant and has the least drug interaction, patients have quick relief of symptoms related to acidity as they have higher supprerssion of the acidity with minimum side-effects. After a long time it is heartening to see availability of a new drug for acid peptic disease with better safety and efficacy profile.